

Two Cases of Injection-Site Reactions Associated with Zolapamine: A Case Series

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Abstract. We present two unrelated fictional cases of delayed injection-site reactions following subcutaneous administration of Zolapamine (fictional biologic). Both patients developed localized erythema and induration beginning several days after injection, with differing severity and management courses. These cases illustrate the spectrum of local reactions that can occur with this product class.

Note: this article describes two distinct, unrelated patient cases, presented sequentially below.

Case 1

A 29-year-old fictional female patient with rheumatoid arthritis began subcutaneous Zolapamine 50 mg weekly. Five days after her third injection, she noticed a 4 cm area of erythema, warmth, and mild swelling at the injection site on her thigh. She reported no fever or systemic symptoms.

On examination, the site showed well-demarcated erythema without fluctuance or drainage, consistent with a delayed local hypersensitivity reaction rather than infection. She was treated with a topical corticosteroid and instructed to rotate injection sites more consistently. The reaction resolved over 10 days, and she continued Zolapamine therapy without recurrence at the six-month follow-up.

Case 2

A 61-year-old fictional male patient with plaque psoriasis received his first dose of Zolapamine 75 mg. Three days later he developed a larger, more painful area of induration (approximately 8 cm) at the abdominal injection site, accompanied by low-grade fever (38.1°C) for one day.

Given the fever and larger reaction size, the treating dermatologist evaluated for possible cellulitis; ultrasound of the site showed soft-tissue edema without abscess, and blood cultures were negative. The reaction was classified as a more pronounced injection-site hypersensitivity response. Zolapamine was held for one cycle, and the patient was restarted at a reduced initial dose with premedication, without recurrence.

Discussion

These two cases demonstrate meaningful variability in the severity of injection-site reactions to Zolapamine, ranging from mild self-limited erythema to a more pronounced reaction requiring evaluation for infection. Clinicians should counsel patients on expected injection-site reactions and maintain a low threshold to evaluate for infection when fever or a rapidly enlarging site is present.

References

1. Sundstrom K, Alvi R. Local tolerability of subcutaneous biologics in dermatologic indications. *Fictional Dermatol Ther.* 2025;14(1):55-61.
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